BERNSTEIN
SOCIETE GENERALE GROUP

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US Biopharmaceuticals

US Biopharma: Can pharma's premium multiples hold after the blue wave?



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The midterms brought a blue wave this week, as broad-based Democratic gains delivered control of both the House and Senate. Since then, we've been getting questions from investors: *Can pharma multiples persist at these levels given the Democrats' focus on drug pricing? Why hasn't the sector retreated?* This note aims to evaluate the trajectory of pharma momentum & different scenarios that might play out.

Does a blue wave end pharma's rerating? We don't think the election result breaks the current investment case. The most immediate risk is that uncertainty associated with policy proposals unsettles generalist investors who have returned to the sector this year. However, we are confident that earnings are unlikely to be impacted as a result of this mid-term election.

More bark than bite. Democratic congressional control is expected to bring greater scrutiny of MFN agreements and may enable opportunities to pursue pharma-focused concessions through funding negotiations. However, presidential veto power and Senate voting requirements will constrain Democratic ambitions for sweeping legislation. We do want to flag that the Sep '27 user-fee reauthorization provides an opening for some regulatory bargaining but believe that the current MFN agreements signed in Nov 2025 continue to provide air-cover until the next term.

The longer-term agenda signals come before the primaries. While the primaries will enable validation of the leading policies & leaders, we already have some hints: Democrats' consultation papers explore negotiating IRA drug pricing earlier, incorporating international prices, changing blockbuster orphan protections and finding a pathway to lower commercial-market prices. Broader coverage (supported by Dems) offers a potential volume offset, but the balance of impact depends on the specifics - net pricing, payer mix and implementation. Detailed bills and committee priorities in 2027 may provide the first meaningful signals.

History suggests caution, but we think the floor is supported. Our election study shows weaker average pharma performance against the broad market following Democratic gains, but individual years illustrate substantial dispersion and a much smaller distinction compared with other traditional defensive sectors. We believe if investors can stomach the "bark" from Democrats, earnings are not at risk & the AI-defensive flows to the sector will continue to support the re-rating that has been established over 2026 (now 17.8x forward P/E).

See the Disclosure Appendix of this report for required disclosures, analyst certifications and other important information. Alternatively, visit our Global Research Disclosure Website.

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BERNSTEIN TICKER TABLE

			4 Nov 2026		TTM	Adjusted EPS				Adjusted P/E (x)	
Ticker	Rating	Cur	Closing Price	Price Target	Rel. Perf.	Cur	2025A	2026E	2027E	2025A	2026E	2027E
ABBV (AbbVie)	M	USD	294.83	275.00	28.4%	USD	9.98	13.93	15.70	29.5	21.2	18.8
AMGN (Amgen)	M	USD	478.48	400.00	53.1%	USD	21.85	23.31	23.53	21.9	20.5	20.3
BMY (Bristol-Myers Squibb)	M	USD	64.19	61.00	32.6%	USD	6.15	6.51	6.54	10.4	9.9	9.8
LLY (Eli Lilly)	O	USD	1,417.99	1,450.00	48.2%	USD	24.20	36.22	54.89	58.6	39.2	25.8
GILD (Gilead)	O	USD	174.48	175.00	33.7%	USD	8.15	(0.73)	10.10	21.4	(237.8)	17.3
MRK (Merck)	M	USD	150.36	105.00	71.1%	USD	8.97	2.56	9.05	16.8	58.7	16.6
MRNA (Moderna)	M	USD	258.38	45.00	979.7%	USD	(7.27)	(8.29)	(3.97)	(35.5)	(31.2)	(65.1)
PFE (Pfizer)	M	USD	30.79	29.00	18.5%	USD	3.23	3.04	2.89	9.5	10.1	10.7
SPX			7,353.73									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

Source: Bloomberg, Bernstein estimates and analysis.

INVESTMENT IMPLICATIONS

We rate LLY and GILD Outperform, and AMGN, ABBV, BMY, MRK, MRNA and PFE Market-Perform

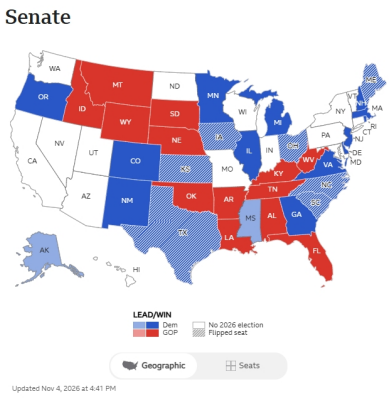
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DETAILS

MIDTERM RESULTS - FEDERAL

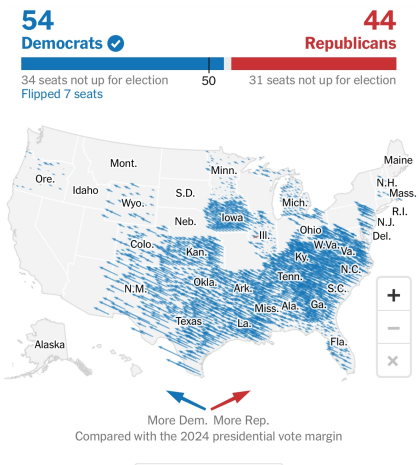
While the blue wave was broad & deep, healthcare doesn't appear to the top issue (according to exit polls) & Trump plans to play the veto card. We would also highlight that political strategists are signalling that this midterm was clearly a rebuke of the current administration's priorities & actions, but may not have been a clear vote for Democrat policies specifically.

EXHIBIT 1: **The Dems flipped the Senate - with key wins in Texas & South Carolina**



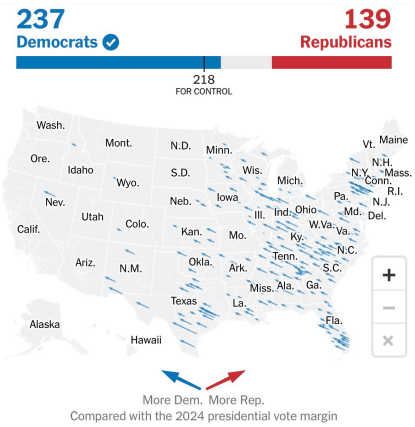
Source: NYT Poll reporting

EXHIBIT 2: **Dems are expected to have at least 54 seats in the Senate**



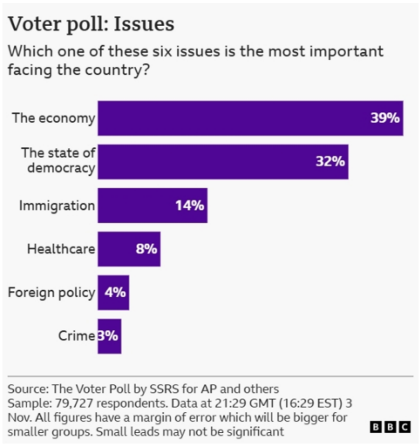
Source: NYT Poll reporting

EXHIBIT 3: **Dems have won the house - some estimates suggest upto ~250 seats might be blue**



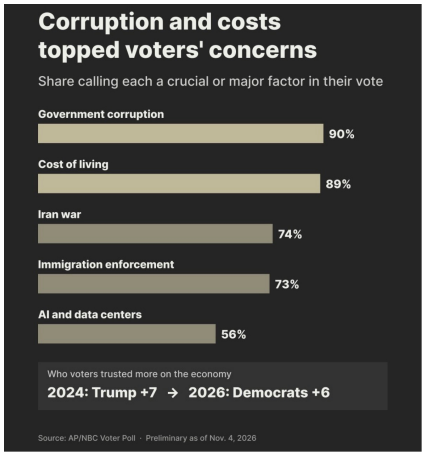
Source: NYT Poll reporting

EXHIBIT 4: **The economy, democracy were apparently top issues for voters**



Source: BBC Poll reporting

EXHIBIT 5: **Corruption & cost of living were major drivers in this poll of voters**



Source: NBC Voter Poll

EXHIBIT 6: **While Dems have gained majorities in two branches, Trump still holds veto power (& plans to use it)**



Source: X

MIDTERM RESULTS - STATE LEVEL

While we don't yet have the full details on the state legislation & Governorship outcomes, there are a series of key states that Democrats have flipped, particularly in more traditionally conservative regions of the country. While these results don't contribute to federal level outcomes, we anticipate that this shifts could be supportive to maintaining & preventing further

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narrowing of Medicaid access.

EXHIBIT 7: **At the State level - we also see the Democrats flipping key Governorships**

State	Outcome	Expected governing position	Medicaid access
Nevada	Dem flipped Governor	Democratic trifecta	Expanded Medicaid adult coverage still in place
Texas	Dem flipped Governor	Divided government	No full expansion was adopted, work requirements do not apply to current program
Georgia	Dem flipped Governor	Divided government	Limited coverage & requires 80h of work/equivalent per mo required
Ohio	Dem flipped Governor	Divided government	Expanded Medicaid adult coverage still in place, however from Jan 1 work/equivalent required
Iowa	Dem flipped Governor	Divided government	Expanded Medicaid adult coverage still in place, however from Dec 1 80h of work/equivalent per mo required

Source: Ballot estimates on Nov 11, State webpages, Bernstein analysis

WITH THE THREAT OF VETO OUT THERE, WHAT CAN THE DEMOCRATS FEASIBLY DO WITH THIS NEW MAJORITY?

We expect noise levels to rise from January through November 2028, but action to remain limited until the next administration. Given the calendar between the mid-terms and next full election, we beleive the likely progression will be as follows:

- **Late 2026–early 2027:** Assuming Democratic control is confirmed, committee appointments and oversight priorities should reveal whether MFN disclosure and drug pricing become formal investigations. Hearings, subpoenas and confirmation proceedings offer opportunities to demand information, while any unresolved funding legislation provides an early bargaining point.

- **February–September 2027:** The administration's budget request, congressional budget debate and appropriations negotiations create recurring opportunities to challenge healthcare cuts, scrutinize MFN savings and seek disclosure requirements or funding restrictions. Budget resolutions can signal priorities without presidential approval, but cannot change law; and reconciliation legislation remains subject to veto**.** The September 30 funding deadline, and subsequent continuing resolutions if needed, provide negotiating pressure.

- **September 2027 is an additional pharma catalyst:** prescription-drug user-fee authority expires & requires reauthorization. It's possible that PDUFA VIII could become a vehicle for regulatory-accountability and transparency provisions (including focused on MFN), although controversial additions could threaten agreement.

- **Throughout 2027–2028:** Investigative findings, detailed bills and presidential candidates' healthcare plans should help us to distinguish shared priorities from factional proposals. Early nominating contests (Jan/Feb) will narrow the field, while the 2028 budget cycle repeats the funding confrontations during the campaign.

WHAT MIGHT THE DEMOCRATS WANT TO DO ON PHARMA & HEALTHCARE?

The clearest public articulation of Democrats' emerging healthcare agenda is the combination of their April 2026 healthcare framework and subsequent policy papers on drug pricing, insurance coverage and long-term care, supplemented by their MFN disclosure proposals and the existing party platform.

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CORE SOURCES AND WHAT EACH CONTRIBUTES

1. **Democratic Vision for Health Care, April 28, 2026.**
   The overarching Senate Democratic healthcare framework: focuses on lower premiums, deductibles, copays, medical care and drug costs across public (Medicare, Medicaid, ACA) and private insurance; aims to improve access - simplifying enrollment and care delivery.

2. **Commonsense Policy Options to Lower Drug Prices for Patients, June 16, 2026.**
   The detailed Senate Finance Committee Democratic drug-pricing request for information. Explores negotiating more drugs earlier, incorporating international prices, finding a pathway to lower commercial-market prices, revisiting blockbuster orphan exemptions, supporting biosimilars, addressing supply-chain incentives and strengthening biopharmaceutical innovation.

3. **Health Coverage That Works for Everyone, July 30, 2026.**
   The Senate Finance Committee Democratic health-coverage request for information. Examines reversing coverage cuts, lowering premiums and deductibles, offering Medicare-like coverage choices, simplifying enrollment, restricting practices that delay or deny care, and addressing corporate arrangements that increase costs.

4. **Drug Deal Disclosure Act: legislative text and one-page summary, released April 21, 2026.**
   A specific proposal to require disclosure of MFN and related manufacturer agreements, including associated direct-purchasing and tariff provisions, alongside independent analysis of their economic and budgetary effects.

5. **Trump's Big Pharma Giveaway, April 21, 2026.**
   The Senate Finance Committee Democratic report accompanying the disclosure proposal. Sets out Democrats' criticism of voluntary manufacturer agreements and their preference for statutory pricing reforms.

6. **Major Overhaul of Long-Term Care in America, October 16, 2026.**
   The official release introducing the Senate Finance Committee Democratic long-term-care request for information. Less relevant for pharma, but covers an affordable home-care guarantee, nursing-home quality and workforce investment.

7. **2024 Democratic Party Platform, 2024.**
   The formal, party-wide baseline for healthcare and prescription-drug priorities, albeit out-dated.

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EXHIBIT 8: **Democratic Vision for Healthcare**

Democrats are focused on lowering costs and making health care actually work for people.

Last week, Leader Schumer laid out how Republicans have slashed health care and inflicted devastating harms on Americans nationwide to fund tax breaks for billionaires and big corporations—and families are paying the price:

- **15 million** Americans are getting kicked off of health insurance
- **100 million** are already in medical debt—with 5 million *more* at risk
- **750+** hospitals, nursing homes, and clinics are facing closure

Right now, families are getting crushed with higher premiums, soaring drug prices, and more barriers to getting basic care.

Republicans' policies are making it worse: siding with wealthy companies, ripping away coverage, and blocking cost relief.

Democrats are taking a different path: putting patients over profit, bringing down out-of-pocket costs, and building a system where getting care doesn't mean going broke.

This is a defining moment. Democrats want a health care system that makes it easier for all families to pursue a healthy life without breaking the bank.

Democrat's Agenda

1. LOWER COSTS — PERIOD

Health care costs are the number one concern for American families.

- **Nearly half of Americans** struggle to afford care
- **4 in 10 adults** carry medical debt

Democrats are focused on lowering costs everywhere:

- **For everyone** — Medicare, Medicaid, ACA, or private insurance
- **For every out-of-pocket cost** — premiums, deductibles, and copays
- **For every kind of care** — prescription drugs, doctor visits, procedures

Bottom line: Health care shouldn't bankrupt you.

Source: Senate websites

EXHIBIT 9: **Democratic Vision for Healthcare (part 2)**

2. RELIABLE CARE

Coverage doesn't matter if you can't actually get care.

Right now, access is shrinking—especially in rural communities—and the workforce is stretched thin.

Democrats are focused on making care accessible:

- **In every community** — rural, urban, and everywhere in between
- **With a strong workforce** — doctors, nurses, and medical professionals nationwide
- **Without interference** — decisions belong to patients and doctors, not to politicians or corporations

Bottom line: If you need care, you should be able to get it.

3. SIMPLER — LESS HASSLE FOR CARE

The system is way too complicated—and Republicans are adding more red tape.

- **Over 70% of Americans** say the system isn't meeting their needs
- **More than a third of insured Americans** do not know what costs their plan covers or what costs they will owe

Democrats will make health care easier to use:

- **Easy to get covered** — simple, clear choices for health insurance plans
- **Easy to stay covered** — no games, no sudden loss of care
- **Easy to get care** — fewer delays, fewer denials, and more provider options
- **Easy to get help** — real support when you need it

Bottom line: Health care shouldn't feel like a maze.

Republicans broke the health care system to benefit the wealthy.

Democrats want to rebuild a system that better serves all Americans.

Source: Senate websites

KEY QUESTIONS OUTSTANDING FOR THE NEXT 2+ YEARS

As foreigners in the US, we are most certainly not the experts on the likely political evolution for the country. However, the key questions below are the most critical in determining is there real pressure likely to be exerted on pharmaceutical companies as we go beyond this administration, or is this likely to just be a step-up in noise, with no real impact to the sector (similar to 2025).

Will the Democrats be led by a Centrist or Leftist? The midterms wave included wins by each centerists & democratic socialists, whether the party is unified & behind what primary policies is yet to be determined.

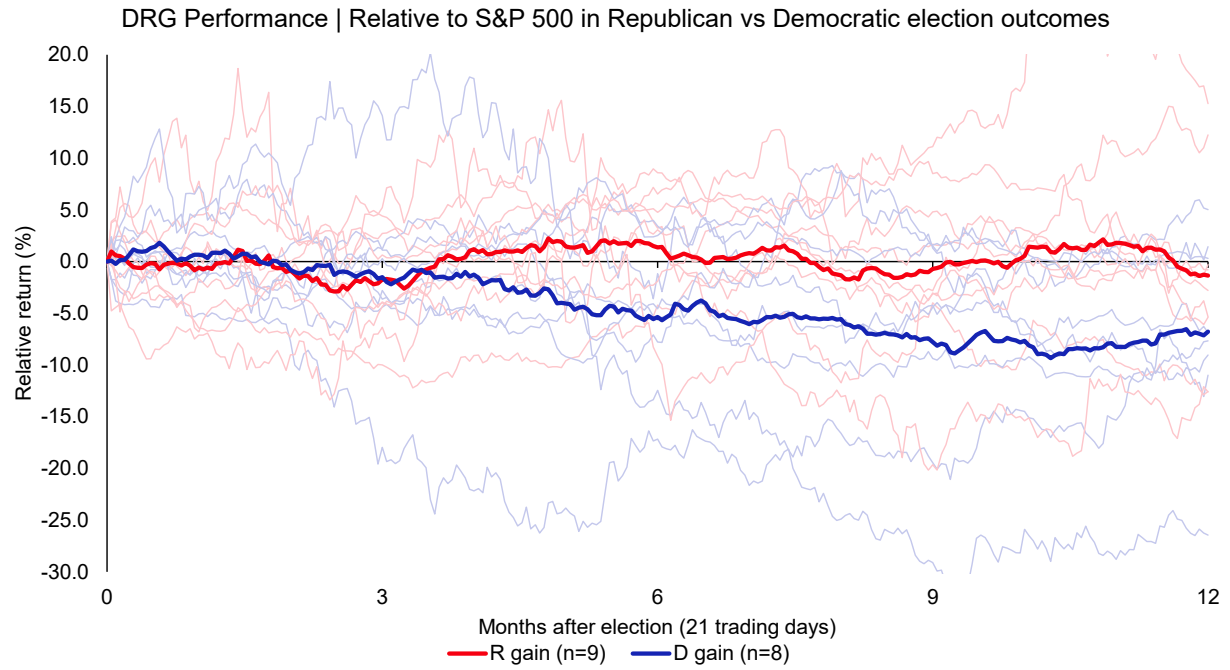
Does the blue wave build? Do we see the results of this election built upon in 2028 & do Democrats hold all three branches of government, or are we more likely to see a split government, ensuring any President has meaningful checks on power.

AFTER AN ELECTION - HOW HAVE PHARMA STOCKS TRADED?

Historically (on average, since 1990), we've seen pharma stocks (DRG) generally perform better under Republican gains than Democrat gains. However, there is extremely wide variation in outcome between elections, with some post-Democrat gain periods translating to more positive outcomes than years with Republican-gains.

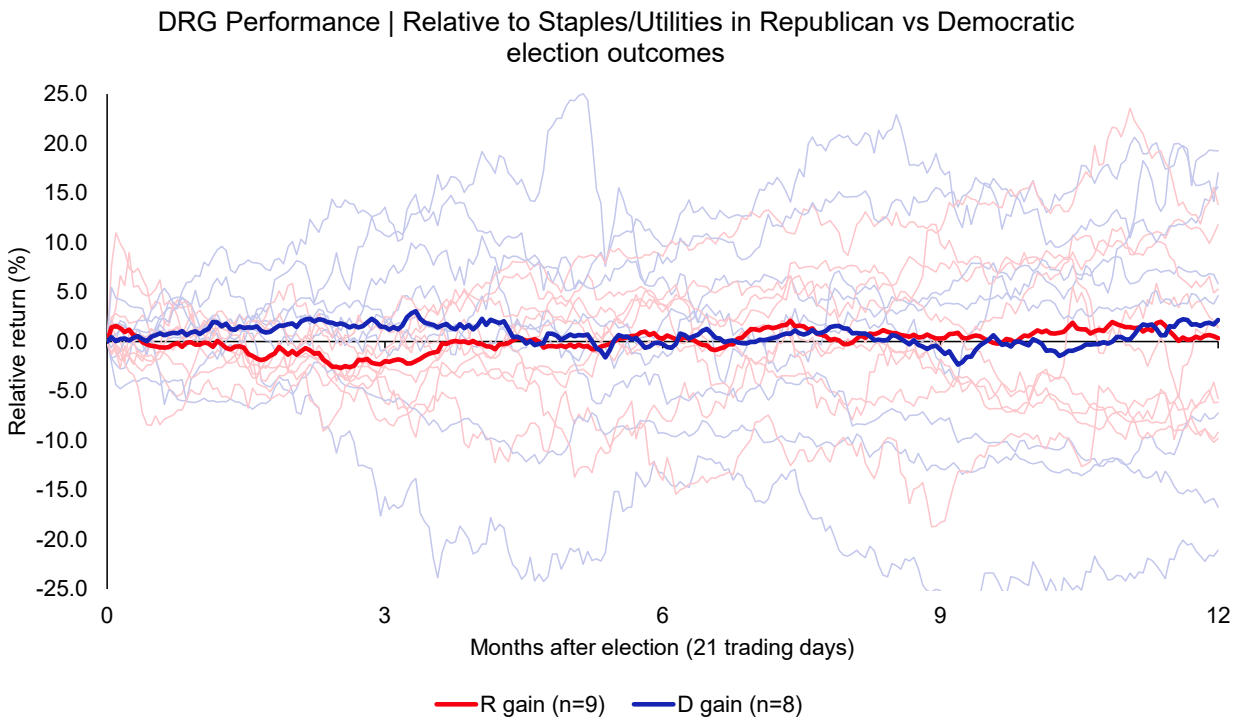
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EXHIBIT 10: **The DRG on average underperforms the S&P after a Democrat election gain - although with notable dispersion**



Source: Bloomberg, Bernstein Analysis

EXHIBIT 11: **The DRG on average performs broadly in-line with defensive sectors like utilities and stapes after a Democrat election gain, although - again - with notable dispersion**

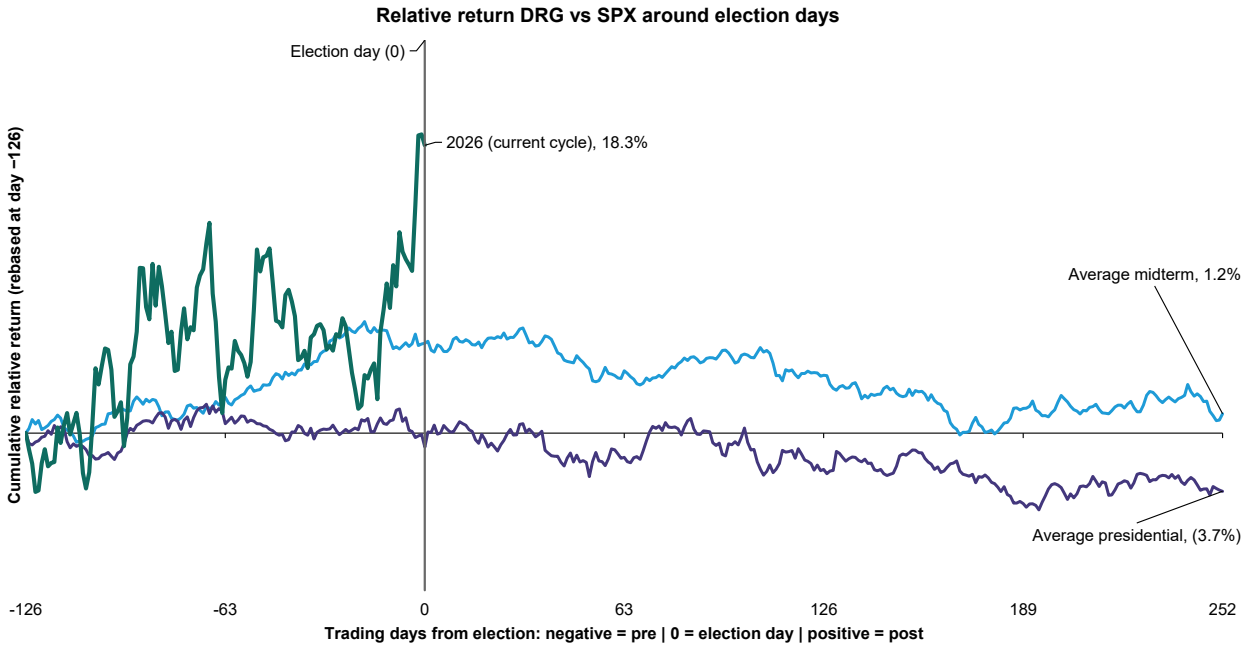


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HOW WAS PHARMA POSITIONED GOING INTO THE ELECTION?

Over the past six-months, the pharma sector has re-rated, with a rise in the DRG of ~18%. This has translated to the current DRG multiple reaching 17.8x, 1.5x below the 20-year high (8.1x-19.3x range).

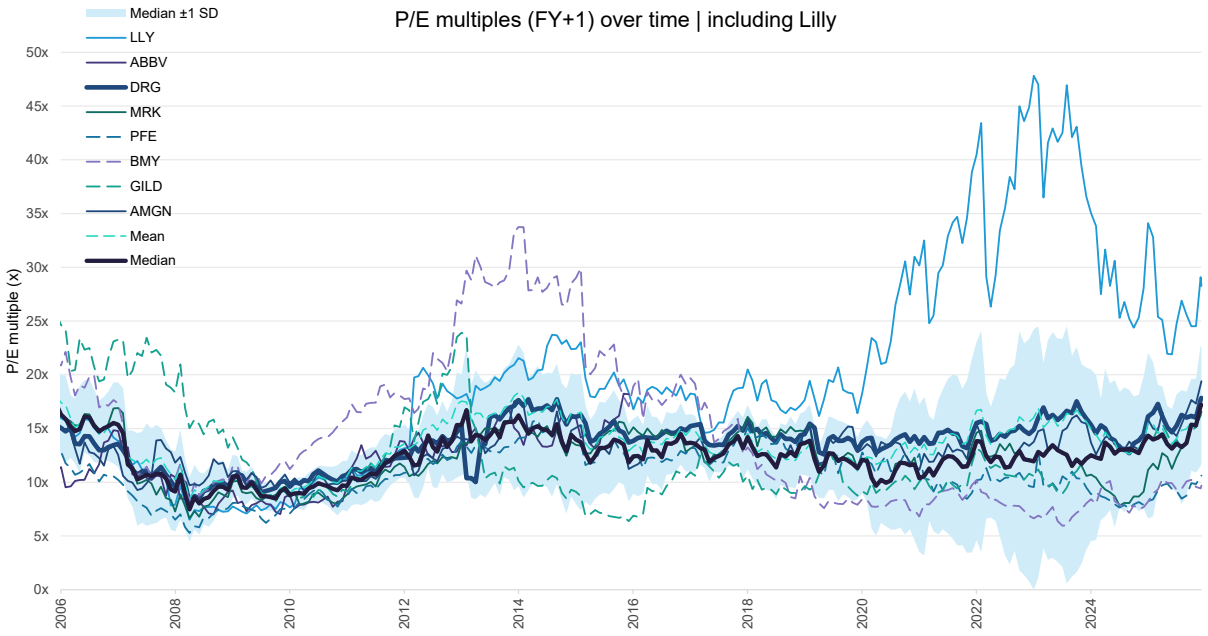
EXHIBIT 12: **The pharma sector (DRG) has had a strong run up into the mid-terms - beyond what we would expect from the historical average**



Source: Bloomberg, Bernstein analysis

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EXHIBIT 13: **Coverage P/E multiples over the last 20 years - current rally shows the sector on the way to all time highs**



Source: Bloomberg Bernstein analysis

WHAT'S DRIVEN THE RE-RATING FOR PHARMA? (IN OUR OPINION)

1. Policy overhang from the current administration was removed in Q4 2025, supporting a revision of the multiple to16.1x (**+1.7x** from Nov 2025-Feb 2026)

2. Generalist investors have re-engaged in pharma, supporting a revision of the mulitple to 17.8x (**+1.7x** from Feb 2026 to current). We believe that has been supported by pharma becoming a more enticing diversificationchoice relative to other non-AI sectors.

    - The caps are large enough to accomodate flows & trade relatively defensively to AI (Exhibit 17)
    - Innovation breakthroughs (like MRNA/MRK's INT) have resonated with investors
    - Pharma's earnings algorithm is unlikely to be disrupted by AI success/failure over the next 5-years, while over the longer term the sector could be an AI winner

AND HOW DURABLE IS THAT RE-RATING?

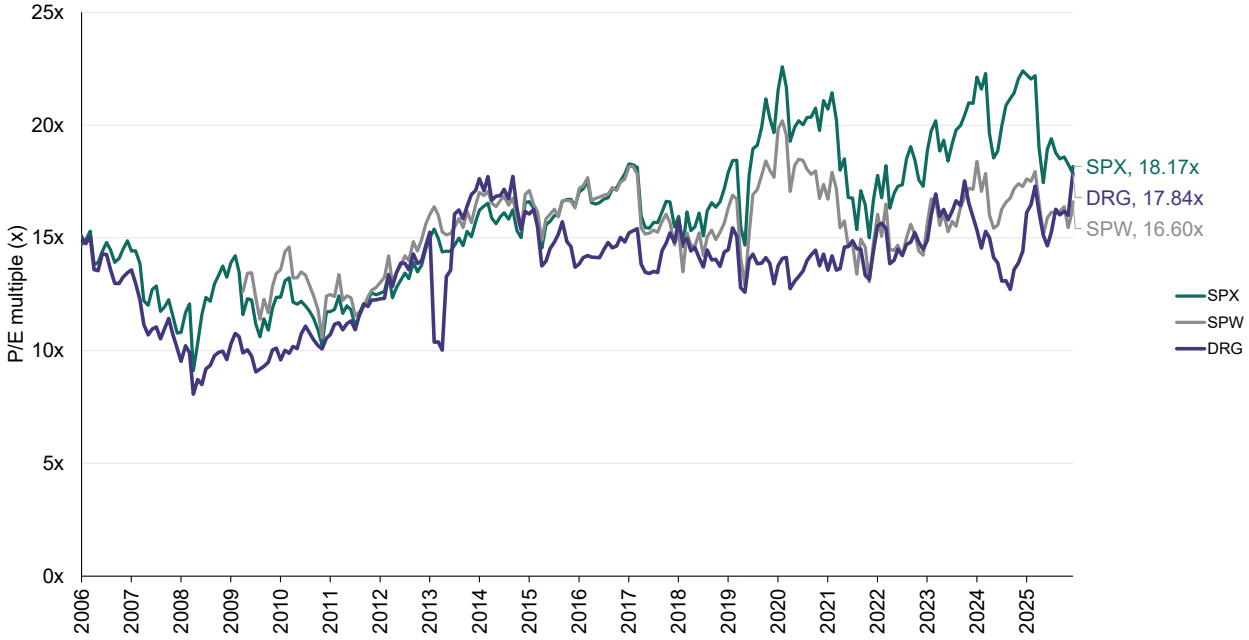
1. While the Democrats are likely to be more directly punitive for drug pricing should there be a future blue-trifecta, it is is not a foregone conclusion that the blue wave builds & we think the next two years are likely to bring more bark (investigations, policy papers) than bite. Real signals regarding the potential for the blue-trifecta and prioritized policies aren't going to solidify until the Primaries in early 2028 (look out for some early signs in 2027). The current (MFN) policy agreements with the administration also specifically preclude any further policy pressure during this term, providing some contractual air-cover. Here is where investors need to learn from 2025 - the noise might be loud, but the sector economics are unlikely to be disrupted over the next 12-24months. If generalists do get nervous, we'd consider a 1x-1.5x reduction in P/E multiple representing the down-side risk.

2. Importantly, we don't believe that pharma becomes any less defensive relative to AI and perhaps this defensiveness might be more highly valued with potential AI-agenda constraints from Democrats at the State level. Relative to other non-AI

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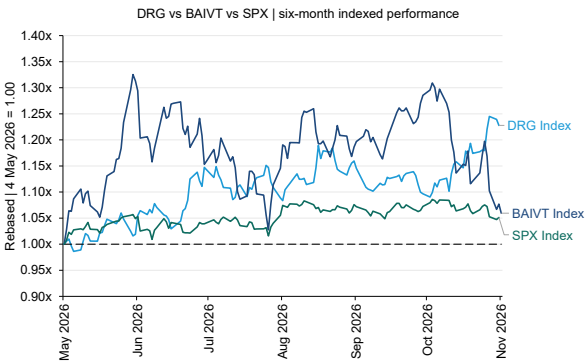
value-chain sectors, we don't see Pharma meaningfully dropping down the ranks for consideration by investors. Pharma maintains it's intrinsic qualities of near-term AI immunity with a growing possibility of a long-term AI tailwind.

EXHIBIT 14: **P/E (FY+1) multiples over last two decades**



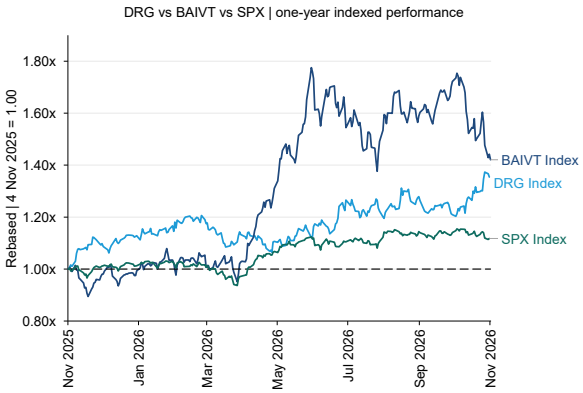
Source: Bloomberg, Bernstein Analysis

EXHIBIT 15: **Six-month pharma outperformance versus the AI basket (BAIVT Index) is consistent with rotation out of AI**



Source: Source: Bloomberg, Bernstein analysis.

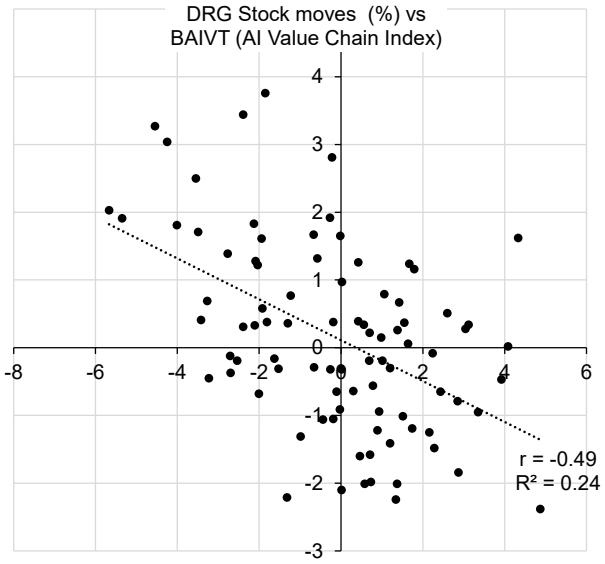
EXHIBIT 16: **Recent pharma outperformance is consistent with rotation out of AI**



Source: Source: Bloomberg, Bernstein analysis.

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EXHIBIT 17: **Pharma has traded inversely to the AI value chain**



Source: Bloomberg, Bernstein analysis

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- Market-Perform: Stock will perform in line with the market index to within +/-15 pp
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